

Surgery for gallbladder cancer

You may have surgery to remove the gallbladder. If the cancer has spread outside the gallbladder, you may have a larger operation called a radical cholecystectomy.

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About surgery for gallbladder cancer

Your doctor will refer you to a specialist liver and gallbladder surgeon if:

- tests show you may have gallbladder cancer
- cancer has been found by chance during surgery.

If the [gallbladder cancer](#) is at an early stage, the aim of treatment is usually to cure it. You may only need to have surgery to remove the gallbladder. This is called a cholecystectomy.

Most people need to have a larger operation called a radical cholecystectomy.

Your doctor or nurse will explain what to expect and how to prepare. We have information about [preparing for surgery](#).

When cancer is found during gallbladder surgery

Sometimes gallbladder cancer is found during surgery to remove the gallbladder because of gallstones. If this happens, you might need further surgery to try to make sure all the cancer has been removed.

If cancer is found during keyhole surgery to remove the gallbladder (laparoscopic cholecystectomy), the surgeon will stop the operation. You will then be referred to a doctor who is a gallbladder cancer specialist.

Radical cholecystectomy

In this operation, the surgeon removes:

- the [gallbladder](#)
- part of the liver
- some [lymph nodes](#).

If the tumour has grown through the gallbladder wall and into a nearby organ, the surgeon may remove all or part of this organ as well.

After your operation, you will stay in an intensive care ward for 1 to 2 days. You will then be moved to a general hospital ward until you recover. Most people need to be in hospital for about 7 to 10 days after the operation.

After surgery to remove your gallbladder

After the gallbladder has been removed, some people have problems with:

- bloating
- wind
- diarrhoea.

These problems usually improve within a few weeks. Tell your doctor or nurse if you have any of these problems. They can give you advice and medicines to help.

We have more information about coping with [bladder and bowel problems](#). Your doctor and nurse will be able to tell you more about the operation and what to expect afterwards.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our gallbladder cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Vogel, A. et al. Biliary tract cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. ESMO Annals of Oncology. 2022. 34,2; 127-140. Available at: pubmed.ncbi.nlm.nih.gov/36372281/ [accessed March 2023]

Fox, E. Gallbladder Cancer. Medscape. Available at: emedicine.medscape.com/article/278641-overview?form=fpf [accessed April 2023]

Mehrotra, B. Gallbladder cancer: Epidemiology, risk factors, clinical features, and diagnosis. UpToDate. 2022. www.uptodate.com/contents/gallbladder-cancer-epidemiology-risk-factors-clinical-features-and-diagnosis [subscription only; accessed March 2023]

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly

- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help